

## NHS Devon Integrated Care Board Service Specification

|                             |                                                               |
|-----------------------------|---------------------------------------------------------------|
| Service Specification No. : | V1                                                            |
| Service:                    | Devon Population Health Local Enhanced Service                |
| Commissioner Lead:          | Siobhan Cambridge Head of Prevention and Outcomes             |
| Provider Lead:              | Primary Care                                                  |
| Period:                     | 30 <sup>th</sup> September 2026 – 31 <sup>st</sup> March 2028 |
| Date of Review:             | 31 March 2027                                                 |

### 1. Population Needs

#### 1.1 National/local context and evidence base

#### 1.2 Purpose and Strategic Context

The Devon Population Health Local Enhanced Service (LES) is designed to support general practice and Primary Care Networks (PCNs) to address priority population health needs and inequalities, through proactive, targeted, and preventative care delivered closer to home.

The LES focuses on early identification, personalised care, prevention, and coordination, aligning with:

- The NHS Long Term Plan
- The national Core20PLUS5 framework
- Devon Joint Forward Plan
- The NHS shift towards community-based, preventative, and digitally enabled care
- This LES complements core GMS, QOF and DES obligations and supports practices to deliver enhanced population health interventions where unmet need is greatest.
- ICB outcomes on prevention, reduced CVD and respiratory disease, and improved healthy life expectancy
- The NHS ambition for a Smokefree Generation
- Best Start in Life
- Cardiovascular disease (CVD) modern service framework (MSF)

The LES is intended to focus on outcomes as opposed to activity and is the first step in supporting population health improvement. Each element has been designed to offer practices and PCNs the necessary flexibility required to support a shift into preventative interventions.

As a new way of commissioning, through engagement with practices, Devon Collaborative Board and Devon LMC, the LES will be developed and refined for 27/28 and beyond.

### 2 Outcomes

#### NHS Outcomes Framework domains & Indicators

<https://www.wp.dh.gov.uk/publications/files/2012/11/121109-NHS-Outcomes-Framework-2013-14.pdf>

|                 |                                                                                              |   |
|-----------------|----------------------------------------------------------------------------------------------|---|
| <b>Domain 1</b> | Preventing People from dying prematurely                                                     | ✓ |
| <b>Domain 2</b> | Enhancing quality of life for people with long term conditions                               | ✓ |
| <b>Domain 3</b> | Helping people to recover from episodes of ill health or following injury                    | ✓ |
| <b>Domain 4</b> | Ensuring people have a positive experience of care                                           | ✓ |
| <b>Domain 5</b> | Treating and caring for people in a safe environment and protecting them from avoidable harm | ✓ |

### 3 Scope

#### 3.1 Aims and objectives of service

#### 3.2 Overarching Aims

The LES aims to:

Be the first Population Health Management driven LES

Improve health outcomes and reduce inequalities in access, experience and outcomes across Devon

Shift activity from reactive to proactive care

Enable more coordinated, person-centred care

Support a digital-first but not a digital-only approach

Make best use of primary care data, digital tools, and community assets

Supporting Integrated Neighbourhood Teams (INTs) to deliver improved health outcomes using PHM

Be accessible to practices signed up and delivering NHS Health Checks commissioned by Public Health

The principle of "Make Every Contact Count"

Improve early diagnosis of long-term conditions

Reduce avoidable emergency and hospital care

Improve patient experience and continuity

Reduce unwarranted variation across Devon

Support sustainability of general practice

Support neighbourhood development

#### 3.3 Summary:

As a Population Health LES with a focus on targeted patient groups there are gateway requirements to sign up, a mandatory element and a group for practices to select.

#### 3.4

|                                                                                  |                                                                                                    |
|----------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------|
| <b>Gateways:</b><br>Practices must meet these criteria                           | Signed up to One Devon Data Set<br>Signed up to deliver Local Authority commissioned Health Checks |
| <b>Mandatory:</b><br>Practices must deliver                                      | Smoking Cessation and Tobacco Harm Reduction                                                       |
| <b>Choose one of the following groups:</b><br>Practices choice on where to focus |                                                                                                    |
| <b>Group 1:</b>                                                                  | Cardiovascular Disease                                                                             |
| <b>Group 2:</b>                                                                  | Frailty (practises signed up to iCOPE cannot choose this option)                                   |
| <b>Group 3:</b>                                                                  | Children and Young People                                                                          |

To support population health need and delivery at scale, funding can be pooled and used at PCN/neighbourhood or wider level.

#### 3.5 Gateways to access Population Health LES Funding

Before signing up to this LES there are criteria, linked to population health improvement, that must be met and sustained for the duration of the LES:

Practices must be signed up to deliver NHS Health Checks commissioned by their Local Authority. Where

signed up but no health checks have been delivered within the previous six months, working with the relevant Public Health team, a clear plan for delivery will be required

Practices must be fully signed up to One Devon Dataset for the duration of the LES

### **3.6 List size and funding**

#### **List size**

The LES and associated funding are based on the raw GP-registered population rather than a weighted population. This is to provide:

- equity of access to proactive care through a targeted approach
- raw lists reflect the actual number of individuals for whom practices are accountable, making it easier to plan outreach, track coverage, and measure impact at a whole-population level
- weighted formulas, while useful for resource allocation, can obscure true service demand at a local level, and may inadvertently disadvantage areas with lower calculated need but significant unmet or emerging prevention requirements, including rural and coastal communities common across Devon
- risk stratification by practices will ensure interventions addressed within this LES will be a targeted approach

A raw list population approach supports clearer population segmentation, consistent benchmarking across practices, and more straightforward communication with stakeholders, while reinforcing the principle that population health management should proactively reach all registered patients rather than being constrained by useful but not mandated measures of need.

### **3.7 Funding Allocation and Methodology**

Funding through PHM and data is a different approach in Devon and will require development. Funding is intended to be distributed fairly, transparently and a step process will be required to support both practices and increasing emphasis on preventative work in line with national priorities:

### **3.8 26/27**

For 26/27 funding will be calculated using raw list size as of 1 April 2026 with practices submitting: intended risk stratification of patients and targeted percentage of population intended to support

Funding for 26/27 is capped at £500k with practices able to specify their PHM approach in delivering proactive preventative interventions to a targeted proportion of their registered patients.

Submission will be admin-light for all parties and is intended as a bridge to allow funding to be released in 26/27, whilst PHM development ensures a more targeted funding approach.

To allow practices to take necessary steps to meet sign-up to criteria, practices will have until 30<sup>th</sup> September 2026 to sign up to the LES. Following the initial sign-up period practices will not be able to sign up until 1 April 2027. This approach is taken to ensure all available funding is spent within the financial year across priority population health measures.

Where full sign up across Devon is not in place, any unallocated funding will be made available to practices delivering the LES through a fair and transparent process.

Any unallocated funding will prioritise Core20PLUS5 groups.

On sign up, practices will be required to submit a brief summary of intended focus with outcomes required by 30<sup>th</sup> April 2027.

### 3.9 27/28

From 27/28 a blended formula will be in place using:

- 75% registered practice raw list size as of 1 January 2027
- The majority of funding is allocated according to registered practice population.

Using list size as the principal component ensures that funding broadly follows activity and workload while maintaining a simple and transparent allocation methodology.

- 15% Core20PLUS5

A dedicated Core20PLUS5 weighting ensures additional funding is directed towards practices serving populations living within the most deprived 20% of neighbourhoods nationally (IMD deciles 1 & 2).

This reflects the need to reduce health inequalities and improve outcomes for populations disproportionately affected by:

- Smoking-related disease
- Cardiovascular disease
- Asthma in children
- Frailty

Practices serving more deprived populations typically face higher levels of need, increased complexity and lower engagement with preventative interventions. The Core20PLUS5 weighting ensures resources are targeted towards those populations where the greatest health gains can be achieved.

- 10% population aged 75 years and over

A further weighting is applied based on the proportion of patients aged 75 years and over.

Older populations are associated with higher levels of:

- Frailty
- Complexity
- Multimorbidity
- Housebound status
- Carer dependency
- Care planning requirements
- Unplanned healthcare utilisation

Including an age-based adjustment recognises the additional workload associated with caring for older and more complex populations.

It is intended to:

Recognise volume of activity through registered list size

Recognise health inequalities through the Core20 weighting

Recognise clinical complexity through the 75+ population weighting

Support prevention and population health objectives by directing resources towards patients with the greatest need

Avoid excessive administrative burden by using routinely available data

Provide transparency and predictability for practices, enabling effective workforce and service planning

Maintains financial control by allowing the ICB to distribute a fixed annual budget without introducing activity-based claims processes

**3.10** The 75% / 15% / 10% funding formula provides a balanced approach aligning resources with population need, health inequalities and service complexity, while remaining simple enough to implement at scale across

Devon. It supports the strategic objectives of the Population Health LES by ensuring funding follows not only patient numbers, but also the populations most likely to benefit from targeted proactive and preventative intervention.

This approach also provides a low administrative burden in calculating for each practice using an open and transparent methodology with readily available data.

One Devon Dataset Dashboard development is expected to deliver a more targeted approach to meet population health need and will become available to practices.

### **3.11 Priority Population Health Components**

The LES comprises three service components chosen by practices, each targeting a high-impact population health opportunity, with smoking cessation as a cross-cutting mandatory element.

These have been determined as evidence-based and impactful for those experiencing the greatest inequity across age groups. Components also enable practices and/or PCNs to select groupings based on their registered population need through the use of risk stratification.

As PHM develops across Devon it is intended for the LES to develop in line with neighbourhood and place priorities and supported through the One Devon Dataset dashboard, with development co-designed with general practice.

Practices (or on a wider PCN/neighbourhood footprint) can select cohorts within groupings based on their registered population. Success will be determined through outcomes that are measurable and achievable within the LES financial envelope and will not focus purely on activity. Practices will be required to submit how they will stratify patients and specify the intended targeted % of their registered population within their chosen cohort.

Across all three groups, the LES funds:  
 proactive case finding through risk stratification, not reliance on patient presentation. Funding can be used to enhance existing work but cannot be used for work funded through alternative routes such as QOF or the PCN DES  
 targeted recall and prioritisation where deprivation, complexity or non-engagement is higher

## **3.2 Service description/care pathway**

### **Cohorts groups**

#### **Mandatory Smoking Cessation and Tobacco Harm Reduction (Cross-Cutting LES Component requirement)**

##### **Background**

Smoking cessation is considered a core component of a Population Health LES because tobacco use remains one of the leading preventable causes of morbidity, mortality, and health inequalities. Smoking is strongly associated with a wide range of chronic conditions, including cardiovascular disease, respiratory illness, and cancer, which disproportionately affect more deprived communities, thereby widening health inequities.

Embedding smoking cessation into this LES enables proactive, systematic identification and support of smokers across primary care populations, aligning with prevention-focused models that prioritise early intervention and behaviour change. It also delivers a high return on investment by reducing future

healthcare utilisation, supporting NHS sustainability, and improving quality of life at scale.

Integrating cessation support into routine care pathways—such as long-term condition management, maternity services, and mental health care—ensures that interventions reach those most at risk, making it a critical lever for improving outcomes and achieving broader public health goals.

Smoking is a major driver of health inequalities because it is far more prevalent among people living in socioeconomically deprived communities and among certain vulnerable groups, including those with mental health conditions and lower educational attainment. These populations are more likely to start smoking, less likely to quit successfully, and experience greater exposure to tobacco-related harm over their lifetime. As a result, smoking contributes significantly to gaps in life expectancy and healthy life expectancy between the most and least deprived groups. It also exacerbates financial hardship, as a greater proportion of income is spent on tobacco, reinforcing cycles of deprivation.

Because smoking-related diseases—such as COPD, cardiovascular disease, and cancer—occur earlier and more frequently in disadvantaged populations, it places an unequal burden on both individuals and health services. Addressing smoking is therefore one of the most impactful ways to reduce unfair and avoidable differences in health outcomes across populations and why it cuts across all groupings within this LES.

### **Aim**

Smoking remains the single biggest preventable cause of ill health and premature mortality, and a major driver of health inequalities in Devon. Rates of smoking are higher in:

- more deprived communities
- people with long-term conditions
- people with mental health needs
- carers
- households with children exposed to second-hand smoke

This component supports systematic, proactive smoking cessation activity in primary care, embedding tobacco dependency treatment across routine clinical contacts. It embeds proactive identification, brief intervention, and referral for smoking cessation into routine primary care activity.

### **Focus**

- Improve identification and coding of smoking status
- Increase the number of smokers offered evidence-based support to quit
- Maximise use of available smoking cessation resources
- Target high-risk groups where smoking drives inequalities
- Reduce smoking-related CVD, respiratory disease, cancer, and poor pregnancy outcomes
- Reduce second-hand smoke exposure in children

### **Key elements**

Primary care will support smoking cessation through five core actions:

- Systematic identification
- Smoking status recorded and kept up to date
- Household smoking exposure identified, particularly where children are present
- Opportunistic identification during reviews (HIU, asthma, carers, healthy child, hypertension, EOL)
- Brief Advice and Motivation
- Delivery of Very Brief Advice (VBA):
- Ask about smoking
- Advise on the benefits of stopping
- Act by offering support

- Framed as a routine clinical intervention, not judgemental lifestyle advice
- Referral to specialist support
- Local Stop Smoking Services
- Community pharmacy support
- Specialist services for priority groups (e.g. pregnancy, mental health)
- Use of social prescribing where wider support is needed (stress, housing, finances)

Smoking cessation is explicitly embedded within groupings to support:

- Recording of offers, referrals, and outcomes
- Follow up where appropriate as part of wider care planning
- Reinforcement of quitting attempts, even where relapse occurs

#### **Through risk stratification prioritisation will be:**

- People living in Core20PLUS5 communities
- People with or at risk of hypertension and CVD
- Children and young people exposed to household smoking
- People with asthma or chronic respiratory disease
- High intensity users of primary care
- People approaching end of life, where appropriate and patient led

#### **Coding**

- Record Smoking and Vaping Status using SNOMED codes:
  - Current Smoker 77176002
  - Ex Smoker 8517006
  - Non-Smoker (Never Smoked) 266919005
  - Vaper 722499006
  - Ex Vaper 35361000087100
  - Never Vaped 1137690000

### **Group 1 – Cardiovascular Disease**

#### **Cardiovascular Disease Prevention – Hypertension Case Finding**

##### **Aim**

To reduce cardiovascular morbidity and mortality through targeted detection and early management of undiagnosed hypertension using Population Health Management.

To reduce cardiovascular morbidity, premature mortality and health inequalities across Devon through the systematic identification, diagnosis and early management of adults with previously undiagnosed hypertension, using Population Health Management (PHM) methodologies, risk stratification, and targeted outreach to populations at highest risk of adverse cardiovascular outcomes.

To support the objectives of the NHS Long Term Plan, Core20PLUS5 approach, the CVDPREVENT programme, the CVD Modern Service Framework, and NHS Devon's commitment to reducing unwarranted variation in cardiovascular outcomes through prevention and earlier intervention.

##### **Objectives**

Funding is intended to:

- Increase the identification of people living with undiagnosed hypertension
- Support earlier diagnosis and treatment of cardiovascular risk factors before the development of preventable heart attacks, strokes, heart failure, chronic kidney disease and vascular dementia
- Reduce inequalities in cardiovascular outcomes through targeted case finding and outreach to underserved populations
- Use Population Health Management approaches and intelligence available through (once available) the One Devon Dataset (ODD), primary care records and other population health tools to identify cohorts at greatest risk
- Improve equity of access to cardiovascular assessment and management services across Devon's rural and coastal communities
- Support delivery of national ambitions for blood pressure optimisation and cardiovascular disease prevention

### **Focus**

The LES is intended to provide a proactive and systematic approach to hypertension detection that goes beyond routine or opportunistic blood pressure measurement activities delivered through standard primary care services or Patient Pathway Optimisation LES activity.

Practices will need to undertake targeted identification of patients at elevated risk of undiagnosed hypertension, prioritising those most likely to experience poor cardiovascular outcomes or barriers to accessing routine healthcare.

### **Distinction from Routine Blood Pressure Activity**

This LES is specifically designed to support proactive, intelligence-led case finding and should be clearly distinguished from routine blood pressure measurement undertaken through standard primary care contacts, NHS Health Checks, annual reviews or the Patient Pathway Optimisation LES.

Activity claimed under this specification must demonstrate:

- Use of population health management approaches
- Identification through targeted searches and risk stratification
- Focus on underserved and high-risk population groups
- A clear health inequalities reduction objective
- Proactive outreach rather than solely opportunistic assessment

The emphasis of this service is not simply on increasing blood pressure recording, but on finding and managing the people most likely to remain undiagnosed and most likely to experience preventable cardiovascular harm.

### **Priority Populations**

Practices should use a risk-stratified approach to prioritise individuals from the following groups:

#### **Core20PLUS5 Populations**

- Individuals living within the most deprived 20%
- Rural and coastal communities
- Individuals, families and communities adversely affected by differential exposure to wider determinants of health – for example, those with insecure housing or homeless, vulnerable migrants, and those experiencing domestic abuse.
- Individuals with serious mental illness (SMI), learning disability, and/or neurodiversity
- Ethnic groups at increased cardiovascular risk (including, Black African, Black Caribbean and South Asian populations)
- Carers

#### Additional High-Risk Clinical Groups

Where identified through PHM searches, practices may also wish to prioritise patients with:

- Obesity
- Chronic kidney disease
- Diabetes or pre-diabetes
- A QRISK score suggesting elevated cardiovascular risk
- Recorded elevated blood pressure readings without formal diagnosis
- Have not attended routine health reviews for a prolonged period
- A family history of premature cardiovascular disease

#### Population Health Management and Case Finding

Practices will use population health tools, risk stratification methodologies and clinical system searches to identify adults aged 18 years and over who may have undiagnosed hypertension.

Searches may include patients with:

- Previously elevated blood pressure readings
- Repeated borderline readings
- Missing or outdated blood pressure records
- Clinical risk factors associated with hypertension
- High cardiovascular risk profiles identified through PHM tools
- Membership of priority health inequality cohorts

Practices should evidence a proactive approach to identifying those least likely to present through routine care pathways.

#### Patient Engagement and Outreach

Practices should use tailored and proportionate approaches to invite identified patients for assessment.

This may include:

- Telephone, SMS, letter and digital invitations
- Outreach through community services and voluntary sector partners
- Integration with Annual Health Checks, Learning Disability Reviews and SMI Reviews
- Reasonable adjustments for patients with communication, cognitive or accessibility needs

Practices should seek to minimise barriers to access and maximise uptake amongst underserved populations.

#### Blood Pressure Assessment and Diagnosis

Confirmation of hypertension must be undertaken in accordance with current NICE guidance.

This should include:

- Clinic blood pressure measurement using validated equipment
- Ambulatory Blood Pressure Monitoring (ABPM) or Home Blood Pressure Monitoring (HBPM) where clinically indicated
- Appropriate coding and recording of results within the clinical record
- Evaluation of cardiovascular risk factors and associated comorbidities

Practices should ensure robust pathways exist for confirmation and coding of diagnoses.

#### Diagnosis and Clinical Management

Patients diagnosed with hypertension should:

- Be added to the practice hypertension register
- Receive cardiovascular risk assessment in accordance with national guidance
- Be offered evidence-based treatment and lifestyle interventions
- Be included in routine long-term condition monitoring and review pathways
- Be managed in line with NICE guidelines and local Devon cardiovascular pathways

### **Health Inequalities Monitoring**

Practices should monitor delivery and outcomes by key demographic characteristics where data quality permits, including:

- Deprivation quintile
- Ethnicity
- Age
- Sex
- Serious Mental Illness status
- Learning Disability status
- Health inclusion group status
- Rural and coastal population indicators

This information should be used to understand reach, uptake, diagnosis rates and any emerging inequalities in service delivery.

### **Expected Outcomes**

The LES will contribute to:

- Increased detection of previously undiagnosed hypertension
- Earlier initiation of treatment and risk factor management
- Improved blood pressure control across the Devon population
- Reduced variation in cardiovascular outcomes between population groups
- Improved engagement of underserved populations in preventative healthcare
- Reduction in avoidable cardiovascular events, including stroke, myocardial infarction and heart failure

## **Group 2 – Frailty**

### **Proactive Frailty (not an available to practices signed up to iCOPE)**

Devon-wide proactive frailty shifts from a universal, high-intensity PCN model (set out in iCOPE) to a more targeted, population health-driven frailty system focused on highest-risk groups.

### **Aim**

To identify, prevent, and manage frailty at scale across Devon by:

- Delaying onset and progression of frailty
- Reducing avoidable hospital admissions and care home placements
- Improving independence, wellbeing, and quality of life
- Delivering consistent, equitable frailty care across Devon

### **Key Design Principles (Devon Adaptation from iCOPE)**

The Devon model is:

- Targeted, not universal intensity
- A Digital-first (but not digital-only) + population health approach
- Leverages existing practice, PCN, community & VCSE assets

- Focus on highest-impact cohorts (risk stratified)

#### Target Cohorts (Prioritised)

| Tier             | Devon Approach                           |
|------------------|------------------------------------------|
| Fit & Well       | Light-touch, digital-first prevention    |
| Mild Frailty     | Targeted prevention (high impact cohort) |
| Moderate Frailty | Core intervention group                  |
| Severe Frailty   | Intensive MDT + proactive care           |

#### Core Service Components

##### Frailty Identification & Risk Stratification

- Systematic identification using:
  - eFI / alternative tools
- Population segmentation:
  - Fit / Mild / Moderate / Severe
- Creation of a Devon-wide frailty register to support PHM development

##### Prevention & Early Intervention

##### Fit & Well + Mild Frailty

- Annual Communication (digital-first)
  - Public health messaging:
    - Staying active
    - Staying connected
    - Staying healthy
- Targeted offer (not universal):
  - Social prescribing referrals (risk stratified)
  - Falls and lifestyle interventions

##### Efficiency measure:

- Delivered via centralised comms + VCSE partners
- Minimal clinical input

##### Proactive Care

##### Moderate & Severe Frailty

##### Priority intervention at Devon scale

##### Comprehensive Geriatric Assessment (CGA) imputed into DCCR when functionality is live

- MDT-led (PCN + community services)
- Must include:
  - LTC optimisation
  - Medication review / deprescribing
  - Functional & social assessment
  - Advance care planning

##### Care Planning

- Shared care and support plan across:
  - Primary care
  - Community teams
  - OOH services
  -

##### Scaled expectation:

- ≥1 CGA/week per 30,000 population (reduced from original spec due to funding constraint)

- Risk stratification prioritisation

Investment limited to:

- Enhancing coordination, not full-service expansion

System Integration to support neighbourhood and place

- Supporting MDT working across:
  - PCNs
  - Community providers
  - VCSE
- Standardised data sharing across Devon ICB

Due to existing funding under iCOPE available to some practices, key differences from iCOPE specification require explicit scoping to ensure funding meets requirements. Expected increases will be scoped and agreed for 27/28 as service embeds and iCOPE is evaluated:

- Reduced per-patient funding = reduced specification
- Fewer CGAs (targeted vs universal)
- Greater focus on digital and VCSE leveraged for prevention

Key Outcomes & KPIs:

Coverage & Identification

- frailty register completeness
- moderate/severe frailty receiving CGA
- mild frailty receiving targeted interventions such as strength and balance classes
- patients with shared care plan
- medication reviews

System Impact expected improvements:

- Emergency admissions (frailty cohort)
- Unplanned bed days
- Length of stay
- Care home admissions
- Urgent response times

Metrics will be further developed in line with Modern Service Frameworks and neighbourhood prioritisation.

### **Group 3 – Children and Young People**

#### **Asthma Identification and High-Risk Management**

##### **Aim**

To improve respiratory health outcomes for children and young people through earlier identification of suspected asthma, timely diagnosis, proactive management of high-risk asthma, and reduction of avoidable exacerbations, emergency department attendances and hospital admissions.

##### **Focus**

This specification adopts a population health management approach, using risk stratification and targeted outreach to identify children and young people who experience poorer outcomes due to clinical risk factors, deprivation, housing conditions, environmental exposures and barriers to accessing care. Particular focus should be placed on children and young people who:

- Live within the most deprived 20% of neighbourhoods nationally (CORE20)
- Are recorded as members of a health inclusion group
- Have a learning disability, autism diagnosis or LDA flag
- Are exposed to parental or household smoking
- Live in temporary, overcrowded or poor-quality housing
- Have previously attended urgent care services with respiratory symptoms
- Have poor medication adherence or low uptake of annual asthma reviews

### **Service Requirements**

#### **Active Case Finding and Early Identification**

Practices should undertake proactive case finding to identify children and young people who may have undiagnosed asthma, including those with:

- Recurrent wheeze
  - Repeated presentations with cough or breathlessness
  - Multiple respiratory consultations within a 12-month period
  - Frequent requests for bronchodilator medication
  - Repeat urgent care attendance for respiratory symptoms
- Practices should prioritise outreach and assessment for children living within CORE20 neighbourhoods, those from health inclusion groups and those with an LDA flag

#### **Diagnostic Assessment**

Children identified through case finding should receive appropriate assessment in line with current clinical guidance, including:

- Clinical history and examination
- Objective diagnostic testing where age and clinical circumstances permit.
- Assessment of environmental and lifestyle factors contributing to respiratory symptoms

Practices should ensure diagnostic pathways are an equitable and accessible approach is taken with patients, taking account of communication needs, neurodiversity, family circumstances and health literacy.

#### **Management of High-Risk Asthma**

Practices should maintain a register of children and young people considered at increased risk of poor asthma outcomes.

High-risk cohorts may include those who have:

- Attended an Emergency Department with asthma symptoms in the previous 12 months
- Required hospital admission for asthma
- Received multiple courses of oral corticosteroids
- High short-acting bronchodilator use or poor preventer adherence
- Significant safeguarding, housing or social vulnerability indicators

Eligible patients should receive face-to-face asthma reviews that include:

- Symptom assessment and asthma control monitoring
- Review of inhaler technique
- Medicines optimisation
- Review of environmental triggers and smoking exposure
- Assessment of wider family and social circumstances affecting asthma control

#### **Personalised Care and Self-Management Support**

All children and young people with a confirmed asthma diagnosis should be offered:

- An up-to-date personalised asthma action plan
- Education appropriate to age and developmental stage

- Support for parents, carers and families to recognise deterioration and seek timely help
- Smoking cessation support for household members where appropriate

### **Post-Exacerbation Follow-Up**

Practices should establish systems to identify children discharged following an asthma exacerbation. A clinical review should be offered within 48 hours of notification of discharge where clinically appropriate, with a comprehensive follow-up review completed promptly thereafter. Reviews should include:

- Assessment of recovery
- Identification of contributory factors
- Review of medicines and inhaler technique
- Updating of personalised asthma action plans
- Consideration of whether the child should be added to the high-risk asthma cohort

### **Expected Outcomes**

- Increased identification of children with previously unrecognised asthma
- Improved uptake of asthma reviews among higher-risk groups
- Increased provision of personalised asthma action plans
- Reduction in asthma-related urgent care attendances and admissions
- Reduction in over reliance on reliever medications
- Reduction in inequalities in asthma outcomes between children living in CORE20 communities and the wider population

### **LES Metric**

Practices should demonstrate that:

- At least 50% of children identified through asthma case-finding activity are from CORE20 neighbourhoods, health inclusion groups and/or have an LDA flag; and
- Anticipated 100% reach of identified high-risk asthma patients receive an enhanced review and personalised asthma action plan

## **Targeted Healthy Child Health and Wellbeing Reviews**

### **Aim**

To improve health, wellbeing and future life outcomes for primary school-aged children through the early identification of unmet physical, emotional, developmental and social needs, with a particular focus on reducing health inequalities and supporting children at increased risk of poorer outcomes.

This service is intended to provide a proactive, prevention-focused intervention during the period between existing universal child health contacts, ensuring children and families with emerging needs are identified and supported earlier.

### **Focus**

This service adopts a population health management approach, using risk stratification and targeted outreach to identify children who may not routinely engage with preventative health services and who are at greatest risk of poorer health and wellbeing outcomes.

Practices should prioritise children aged 5–11 years who:

- Live within the most deprived 20% of neighbourhoods nationally (CORE20)
- Are identified as belonging to a health inclusion group
- Have a learning disability, autism diagnosis or Learning Disability and Autism (LDA) flag

- Are known to Early Help, Children's Social Care or safeguarding services
- Are young carers
- Have a long-term condition, including asthma, obesity or neurodevelopmental conditions
- Have repeated non-attendance at healthcare appointments
- Are known to be living in temporary, overcrowded or poor-quality housing

To ensure a clear and measurable inequalities focus, practices should demonstrate that a minimum of 50% of completed reviews are undertaken for children living in CORE20 neighbourhoods and/or children meeting one or more of the additional vulnerability criteria listed above.

## **Service Requirements**

### **Proactive Identification and Outreach**

Practices should use population health intelligence, clinical system searches and local knowledge to identify eligible children who may benefit from an enhanced review.

This should include proactive invitation and outreach approaches for families who may face barriers to accessing healthcare services, including those experiencing deprivation, social exclusion or low engagement with routine care.

### **Holistic Health and Wellbeing Review**

The review should take a whole-child and whole-family approach, considering the broader determinants of health as well as physical and emotional wellbeing.

The review should include assessment of:

#### **Physical Health**

- Growth and healthy weight
- Nutrition and physical activity
- Asthma symptoms and respiratory health
- Sleep and general health
- Oral health promotion
- Management of existing long-term conditions

#### **Emotional Health and Wellbeing**

- Emotional wellbeing and resilience
- Behavioural concerns
- Anxiety and low mood
- Peer relationships and social connectedness
- School engagement where appropriate

#### **Family and Social Circumstances**

- Family wellbeing and parental health
- Housing quality, housing insecurity and overcrowding
- Financial hardship and access to support
- Food insecurity
- Exposure to smoking within the household
- Social isolation and access to community networks

### **Personalised Prevention and Early Intervention**

Where needs are identified, practices should work with children and families to develop an appropriate support plan.

This may include:

- Advice and support on healthy lifestyles
- Weight management interventions
- Asthma assessment or referral where symptoms are identified
- Mental wellbeing support

- Smoking cessation support for household members
- Referral to social prescribing services
- Signposting to local voluntary, community and family support organisation
- Referral into statutory services where appropriate

Interventions should be proportionate to need and designed to support prevention, early intervention and self-management.

### **Social Prescribing and Community Connections**

Practices should maximise opportunities to connect children and families with community assets and local support services that can improve health and wellbeing outcomes.

This may include access to:

- Family support programmes
- Physical activity opportunities
- Community groups
- Welfare and benefits advice
- Housing support services
- Parenting programmes
- Voluntary and community sector organisations

### **Follow-Up and Ongoing Support**

Children with identified concerns should have a documented follow-up plan that is proportionate to need. Practices should ensure appropriate monitoring, review and onward referral where concerns are identified and should maintain oversight of children requiring ongoing support.

### **Expected Outcomes**

- Improved identification of unmet health and wellbeing needs in primary school-aged children
- Increased engagement with preventative healthcare among families experiencing deprivation
- Earlier intervention for emerging physical, emotional and social concerns
- Increased access to community support and social prescribing services
- Improved asthma, healthy weight and emotional wellbeing outcomes
- Reduction in health inequalities experienced by children living in CORE20 communities
- Greater understanding of local child health needs to inform wider population health planning and neighbourhood support.

### **Outcomes**

Practices should demonstrate:

- At least 50% of completed reviews are undertaken with children living in CORE20 neighbourhoods and/or meeting an agreed vulnerability criterion
- 100% of children receiving a review have a documented assessment covering physical health, emotional wellbeing and wider determinants of health
- Evidence of appropriate referrals, signposting or social prescribing interventions where needs are identified
- Key themes, inequalities and unmet needs identified through the reviews proactively used to support population health management and service development across Devon

## **3.3 Population covered**

As defined within each group.

## **3.4 Any acceptance and exclusion criteria**

Across all three groups, the LES funds:

- proactive case finding through risk stratification, not reliance on patient presentation. Funding can be used to enhance existing work but cannot be used for work funded through alternative routes such as QOF or the PCN DES
- targeted recall and prioritisation where deprivation, complexity or non-engagement is higher

Before signing up to this LES there are criteria, linked to population health improvement, that must be met and sustained for the duration of the LES:

- Practices must be signed up to deliver NHS Health Checks commissioned by their Local Authority. Where signed up but no health checks have been delivered within the previous six months, working with the relevant Public Health team, a clear plan for delivery will be required
- Practices must be fully signed up to One Devon Dataset for the duration of the LES

Practices will be required to submit a brief summary of intended focus, with outcomes required by 30<sup>th</sup> April 2027.

### 3.5 Interdependencies with other services

NHS Health Checks  
Voluntary sector

## 4 Applicable Service Standards

Add relevant formulary links/guidance

Frailty - [North & East - North & East](#)

Asthma - [North & East - North & East](#)

Tobacco - [4.10.2 Nicotine dependence - North & East](#)

Frailty - [South & West - South & West](#)

Asthma - [South & West - South & West](#)

Tobacco - [4.10.2 Nicotine dependence - South & West](#)

## 5 Key Service Outcomes

As defined within each group.

## 6 Applicable Quality Requirements

Add links to relevant NICE guidance

[Overview | Hypertension in adults: diagnosis and management | Guidance | NICE](#)  
[Multiple long-term conditions: frailty register | Indicators | NICE](#)  
[Overview | Multimorbidity: clinical assessment and management | Guidance | NICE](#)  
[Overview | Asthma pathway \(BTS, NICE, SIGN\) | Guidance | NICE](#)  
[Overview | Asthma: diagnosis, monitoring and chronic asthma management \(BTS, NICE, SIGN\) | Guidance | NICE](#)  
[Project information | Dupilumab for maintenance treatment of uncontrolled moderate to severe asthma in children 6 to 11 years \[TSID10085\] | Guidance | NICE](#)  
[Overview | Asthma | Quality standards | NICE](#)  
[Overview | Smoking: reducing and preventing tobacco use | Quality standards | NICE](#)  
[Overview | Tobacco: preventing uptake, promoting quitting and treating dependence | Guidance | NICE](#)  
[Smoking: smoking status for people with long-term conditions | Indicators | NICE](#)  
[Smoking: smoking status of people with long-term conditions | Indicators | NICE](#)

## 7 Financial Requirements

In line with SMS tier rates – to be described/negotiated

Practices signed up the LES will receive a one-off payment in line with spreadsheet below in addition to usual contractual funding e.g capitation. Claims will take place via the ICBs commissioning portal once all sign up requirements have been met.

Funding is fixed and paid upfront for 26/27 with an additional payment in 27/28 in line with intended specification as detailed in section 3.10.



Devon\_LES\_Fundin  
g\_Allocation\_500k s

DDRB will be used as the start point for annual 'inflationary' review, with any deviation from DDRB uplift requiring clear rationale.

In the event of a pandemic or other 'level 4 health' event, commissioner and LMC will meet urgently to review this specification giving consideration to:

- suspension of some or all elements of it,
- redirection of patients to alternative provider(s),
- in the event of actioning either a) or b) determining appropriate income protection arrangements for GP provider

## 8 Information Requirements

i.e. reporting requirements

Practices will be required to submit which groups they will target, how and expected outcomes. This will be through

a simple Microsoft Word questionnaire with a required 50 - 150-word narrative.

Practices will be required to submit an outcomes form by 30<sup>th</sup> April 2027.

Where funds are unallocated by 30 September 2026 a simple EoI will be required from practices wishing to access additional funding available from the 26/27 £500k.

Practices not participating in 26/27 will have an opportunity to sign up for 27/28 funding if they meet the necessary criteria.

## **9 Location of Provider Premises**

The provider premises.